PathwayAI | Hudson Valley

Dutchess County Lyme Burden & Action Brief

Public evidence for a practical support discussion · Version 1.3

Prepared: 2026-11-01 · Surveillance: 2024 · Study costs: 2016 dollars

Starting point for a county discussion. Newer official county releases have not been verified for this brief.

1. WHAT THE COUNTY DATA SHOW

Area	Reported cases · 2024	Reported rate per 100,000
Dutchess	1,060	355.5
Orange	964	237
New York State	21,632	109.8

County of residence, not exposure location. Official published rate; exact population denominator and vintage not independently verified.

2. STUDY-BASED ECONOMIC ILLUSTRATION

Calculation · original 2016 dollars	Illustrative amount
1,060 reported cases × USD 2,032	USD 2,153,920

Hook et al. (2022): 613 participants in societal analysis; reported cases recruited in four high-incidence states during 2014–2016. Societal: medical, nonmedical and productivity costs; components are already included.

Applying this study mean to reported county cases is an assumption; the result is not measured local spending. No adjustment for underreporting or local case mix. No causal savings or disability probabilities are inferred.

The published study illustrates scale; it is not a current local cost estimate. Inflation changes purchasing power, not treatment patterns or local representativeness.

LOCAL HEALTHCARE RESOURCES AND LANGUAGE

General healthcare resources · 2023	Count
Primary-care physicians	233
Hospitals	2
Hospital beds	404

HRSA AHRF resource counts do not establish Lyme expertise, current staffing or available appointments. Existing HRSA AHRF 2024–2025 asset; 2023 general resource counts, not appointments or Lyme expertise.

Language other than English at home: 16.2% of residents age 5+, ACS 2020–2024. This is not a measure of English proficiency or interpreter demand.

3. WHAT WE STILL NEED TO UNDERSTAND

Work changes, unpaid care, travel and daily-life limitations matter. The selected study population does not represent every local household. Continuing costs, Lyme-attributable disability and local unmet support needs remain unmeasured here.

From evidence to a support question

4. A SUPPORT QUESTION TO DISCUSS

Care without insurance and medical bills

Question: Could existing services help residents without insurance reach affordable care, or are there gaps that need an additional response?

Possible response: Review income-based health-center fees, help applying for coverage, and hospital financial assistance with residents. Check staffing, appointments, costs and possible funding before proposing a new service.

Who could help: Patients, health centers, hospital financial-assistance staff, enrollment assisters, county planners and potential funders

Proposed actions for local review—not findings about demand, committed partners or a funded service.

5. WHAT TO CHECK BEFORE STARTING A PILOT

- Ask residents and caregivers what help they need. Offer ways to participate by phone or in person.
- Check which services are available, who qualifies, whether interpreters are offered, and whether appointments are available.
- Decide who will do the work, how much staff time and money it needs, and how it could be funded.
- Before collecting information, agree on consent, what is needed, who can see it, how long it is kept, and how it is deleted.

6. CHECK WHETHER PEOPLE RECEIVED SUPPORT

Choose a follow-up period, such as 30 days. Record appointments attended, help with applications, financial-assistance decisions, and needs still unmet, as relevant to the selected support priority. Compare these with each person’s starting situation. Keep unanswered questions marked as unknown. Use the findings to improve support; further evaluation is needed to establish cost savings or health benefits.

For this support priority, check: Appointments attended; help applying for coverage; financial-assistance applications approved, denied or pending; bills and support needs still unresolved over the same follow-up period

WHAT STUDIES AND PATIENT REPORTS ADD

- Work changes, unpaid caregiving, travel and daily-life limitations can affect households beyond medical bills.
- Hook societal costs already contain some of these components; do not add them again.
- Horn (2025) describes selected three-month follow-up participants, not county disability risk. Johnson (2014) describes a selected persistent-illness survey, not all new Lyme cases.
- The project’s voluntary 33-person survey snapshot is across locations; question-level denominators remain unverified. No survey percentages or dollar amounts are extrapolated to this county.

SOURCES AND METHODS

1. Reported cases, county of residence · 2024 · Open source
2. Reported rates per 100,000 · 2024 · Open source
3. Societal study mean · 2016 dollars; recruitment 2014–2016 · Open source
4. General healthcare resources · 2023 fields; AHRF 2024–2025 release · Open source
5. Language spoken at home, age 5+ · ACS 2020–2024 · Open source
6. Published follow-up context · Horn et al. 2025 · Open source
7. Published work and function context · Johnson et al. 2014; 2013 survey · Open source

The source ledger and CSV retain source URLs, reporting periods, review status, calculation assumptions and inflation provenance.

Public evidence and proposed planning actions only. No session stories, personal records or synthetic patient cards are included.

Public-data demonstration. No county or organizational endorsement is implied.